

إِسْمُ الْمَرِيضِ رِبَاعِيًّا : وَلَاءُ حَامِدٍ عَلَى إِبْرَاهِيمَ

المحافظة : القاهرة الحي / المدينة : المطرية

العنوان : زوير

تاريخ تسجيل الإستمارة : 04-10-2026

رقم البطاقة : 28604151700541 رقم التلفون : 01000212793

العمر(تاريخ الميلاد) : 15-04-1986 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني الوظيفة: _____
 الاسم: د/ندى حسن الوظيفة: _____
 الاسم: د/نرمين رمضان الوظيفة: _____

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: ولاء حامد على ابراهيم National ID: 28604151700541	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



طلب علاج أول مرة / متردد

جوستاف روسي انترناشيونال - مصر (EPE) *

إسم المريض رباعياً : ولاء حامد على إبراهيم
المحافظة : القاهرة
الحى / المدينة : المطرية
رقم البطاقة : 28604151700541 رقم التليفون : 01000212793
العمر (تاريخ الميلاد) : 15-04-1986 المهنة : ربه منزل
العنوان : زوير
تاريخ تسجيل الإستشارة : 02-07-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج على نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (.....) (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

قسم العلاج : الأورام

خطة العلاج : علاج موجه فسيجو

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة



التوقيع:

التوقيع:

التوقيع:

الوظيفة:

الوظيفة:

الوظيفة:

الاسم: د/احمد نجيب

الاسم: د/خالد عبدالعزيز

الاسم: د/محمد كيلاني

أطلعت اللجنة على لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د..... /د..... /د..... /د.....

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج :

ملاحظات :

قسم :

نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name:	Walea Hammed Ali Ibrahim	GUSTAVE ROUSSY 1893 - 1968
MR. :	5558	
تقرير طبي		
بشأن تقرير طبي للمريض رقم طبي: تحيية طيبة وبعد.. ايحاء لطلب المقدم من بقسم: نشرف بان نوضح فيما يلي تقريرنا عن حالة تاريخ وساعة : mm/dd/yyyy -- -- -- -- الخروج تاريخ وساعة : mm/dd/yyyy -- -- -- -- الدخول وبعد الكشف والفحوصات والمتابعة وجد انه breast cancer phesgo التاريخ : mm/dd/yyyy الوقت : --:--:-- الطبيب المعالج : بمعد		

Name: <u>ولاء حامد على ابراهيم</u>		
MR. : <u>5558</u>		
Detailed Medical Report		
		السيد
نتيجة طبية وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشر بأن نوضح فيما يلي تقريراً عن حالتها		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمناظرة وجد انها		
The patient is a 39-year-old individual with a newly diagnosed invasive ductal carcinoma (IDC) of the left breast, grade II, estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, HER2 2+ with amplification, clinical stage T2 N1 M0. Neoadjuvant chemotherapy (NACT) with the TCHP regimen was initiated for six cycles, followed by left breast-conserving surgery (BCS). Multidisciplinary team (MDT) discussion recommended adjuvant therapy with Phesgo for up to one year, placement of a PORT, and endocrine therapy with femara and zoladex. The patient presented for cycle 1 of Phesgo (1200 mg/600 mg) and received an echocardiogram showing left ventricular ejection fraction (LVEF) of 65%; femara and zoladex 10.8 mg were continued. The pharmacy recorded that the patient had received seven cycles of Phesgo as part of the NACT regimen and administered cycle 8 of Phesgo (600 mg/600 mg) on the same day. On a subsequent visit, the patient was scheduled for cycle 9 of Phesgo combined with femara; a pre-operative echocardiogram performed in January 2026 showed an LVEF of 62%, and the patient had received only one prior cycle of the current regimen. During the medical oncology follow-up, the patient received cycle 11 of Phesgo together with aramidex (substituted for femara because of severe bony aches). The echocardiogram demonstrated an LVEF of 62% on 22/6. The patient subsequently proceeded with cycles 12 and 13 of Phesgo, continuing aramidex. The pharmacy note on 08/08/2026 confirmed administration of cycle 12 of Phesgo with aramidex, with the echocardiogram on 22/6 still showing an LVEF of 62%. On 29/08/2026 the patient reported chronic fatigue; laboratory evaluation for TSH, vitamin D, and CBC was ordered, and the previously placed PORT was noted as completed. In the most recent oncology note, the patient's left breast cancer is described as clinical T2 N1, IDC, ER 6/8, PR 6/8, HER2 2+ with amplification, having received the TCHP regimen on 28/01/2026, followed by left BCS. The patient is currently receiving cycle 13 of Phesgo together with femara. The patient has completed 13 cycles of Phesgo to date.		
تحريراً في ,		
التاريخ :	21-09-2026	الوقت :
الطبيب المعالج :		Reham Ahmed Fathy Ibrahim
يعتمد ,		

السيد أ.د / مدير عام المجالس الطبية

تحية طيبة وبعد
كشف حساب

علاج على نفقة الدولة

بيان بنفقات المريض / **والد حامد**

عن الفترة من 2026 / 3 / 28 الى 2026 / 6 / 30

البيان	رصيد القرار	قيمة المنصرف
الرصيد	94713	
اقامة	0	
كشف	0	
اشراف طبي	0	
اشعه (رنين - مقطعية - اخرى)	0	
تحاليل طبية	0	
مستلزمات	0	
علاج متنوعه (بنك دم / علاج)	0	
الادويه	0	
المتبقي	299	

المراجع



الموظف المختص



جمهورية العراق
محافظة نينوى
ولاية
حشد علي ابراهيم طعيمة
شروع مسجد النور - نينوا
مركز شين الكوم - النورية

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251

٢٠٢٤/١٠ ٢٨٩٠٤١٥١٧٠٠٥٤١
حظية علي بكور علي
شعبة كيمياء
انتي مسلمة
ضياء احمد رمضان احمد
البطاقة سارية حتى ٢٠٣١/١٠/١٦

١٢

02/28/2026

MDT FORM

Name:	Walaa Hammed Ali Ibrahim	MRN:	5558
Age:	40 Y	ID:	
Menopausal Sta:	PREMENOPAUSAL	Phone:	01068496392
NRS SCORE:		Marital Status:	MARRIED
BMI:		Family History:	-VE FH
CO Morbidity:	MEDICALLY FREE	Reason Of MDT:	POST OP

Clinical Data:

BX:	IDC
ER:	6/8
PR:	6/8
HER2:	+2
KI67:	50%

Sonomammography:

SP LT BCS + SLNB POST NACT
NO RESIDUAL INVASIVE TUMOUR
DCIS FREE
SURGICAL MARGIN FREE
EXTRA MARGIN FREE
SLNB (0/3)
PCR

AKRAM ELBADAWY

Mets Workup:

History:

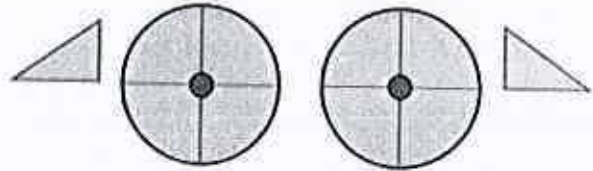
Recommendations:

MDT Outcome & Plan

for adJ PHESGO total of 1 year
PORT
ADJ femara zoladex 7 years

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

000A

Patient Name : الأستاذ / ولاء حامد على ابراهيم	Request No. : 13004745
Patient No. : 28604151700541	Request Date : 28/01/2026
Path No. : 268/26	Verification Date : 08/02/2026
Sex : Female	Financial Category : جوسفاف روسي - نفقة الدولة
Age : 39 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.

Nature Specimen: Left breast conservative surgery, Extra-margin & Sentinel lymph nodes biopsy.

Gross Examination:

* Left breast conservative surgery specimen measured 6.5x03x01 cm, with no skin covering.

* Serialling showed areas of grey white streaks up to 1x0.5 cm within fatty background, located 0.5 cm from the least deep margin. Multisected & totally submitted.

* Extra-margin: Fibrofatty tissue piece measured 5x4x1.5 cm, showed inserted wire, looked unremarkable grossly.

* Sentinel lymph nodes: Multiple fibrofatty tissue pieces collectively measured 3x3 cm, entangling 3 lymph nodes, the largest measured 3x1x0.5 cm.

* 16 Blocks:

1:6- 1st specimen (TS)., 7- Medial margin., 8- Lateral margin., 9- Superior margin., 10- Inferior margin., 11- Superficial margin., 12- Deep margin., 13:15- 2nd specimen., 16- SLNs.

- Gross was done by; Dr. Mennan Hossam.

** Frozen (Intra-operative) Results:

- Free margins.

- SLNs: Free three sentinel lymph nodes, (0/3).

Microscopic Examination:

* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.

- Cellularity (0%).

* Angiolinvasion: Absent.

* Ductal carcinoma in situ: Absent.

* Surgical margins: Free.

* Extra-margin: Free.

Verified by :

Dr.Duaa Mohamed



Patient Name : الأستاذ / ولاء حامد على ابراهيم	Request No. : 13004745
Patient No. : 28604151700541	Request Date : 28/01/2026
Path No. : 268/26	Verification Date : 08/02/2026
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 39 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- * Sentinel lymph nodes: Three free sentinel lymph nodes, SLNs (0/3).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

Left breast cancer (Post-NACT), Left breast conservative surgery, Extra-margin & Sentinel lymph nodes biopsy:

- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Surgical margins: Free.
- Extra-margin: Free.
- Sentinel lymph nodes: Free three sentinel lymph nodes, SLNs (0/3).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature

Prof. Dr. Gamal Abd-El-Aty.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed



Name : WALAA HAMED ALI

Date : 27-12-2025 04:02

Branch : Portable

Medical No. : GRI-28604151700541

Ref.Physician Prof :

Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Findings:

- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- Normal appearance of the dorsal vertebrae.
- Upper abdominal cuts are unremarkable.

OPINION:

- ***Essentially normal CT study of the chest.***

Dr. Mai Seddik

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.

Name : WALAA HAMED ALI

Date : 27-12-2025 04:02

Branch : Portable

Medical No. : GRI-28604151700541

Ref.Physician Prof :

Ins.Company :

- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- ***Normal CT scan of the abdomen and pelvis.***

Dr Mai Mohamed MD

Dr. Mai Seddik

Dr. Mai Seddik

PATIENT NAME	WALAA HAMED ALY
DATE	27 / 12 / 2025

TARGETED AXILLARY ULTRASOUND :

Indication : Evaluation of axillary LNs after NAC of left breast carcinoma.

-Bilateral Non specific axillary LNs .

Much Obliged

Prof . DR.MONA ZAKY

Patient's Name : Walaa Hamed Aly Ebraheem
Date : 27 November 2025
Prof Dr :

BOTH BREAST & AXILLAE SONOMAMMOGRAPHY

Mamographic findings:

- Pattern of breast parenchyma (ACR B).
- Clip is seen inserted at left breast UOQ with no definite lesions.
- No definite masses.
- No definite calcification.
- Normal skin, normal nipple and areola.

Ultrasound correlation:

- Scanning the left breast UOQ: showed no definite residual masses.
- Average size and echogenecity of glandular tissue for patient's age
- No solid or cystic lesions could be noted.
- Normal skin and subcutaneous tissue.
- Normal reteroareolar lactiferous ducts.
- Normal nipple.
- Normal Doppler study of axillary vessels.
- Right axilla showed non specific axillary lymph nodes.
- Left axilla showed lymph node with increased cortical thickness measuring about 19x7 mm (4.2 mm focal cortical thickening)

OPINION

Known case of left breast cancer received neoadjuvant chemotherapy and clip insertion.

- ⇒ Left breast UOQ clip with no definite residual masses.
- ⇒ No definite right breast lesions.....BIRADS-1.
- ⇒ Still noted left axillary lymph node with increased cortical thickness.
- ⇒ Nonspecific right axillary lymphadenopathy.

With our best wishes & regards

Aya Khairat, MSc



Code: 22740
Sex: Female
التاريخ: 1 - 5 - 2025م

اسم المريض: ولاء حامد علي ابراهيم
السن: 38 سنة
الطبيب المعالج: اد/ سوزان الحسين

Clinical Data: Left breast mass.

Pathology report

Nature of the Specimen:
Tru cut biopsy.

Gross Appearance:

Specimen contained three grayish tissue cores ranging: 11-15 mm long, totally submitted.

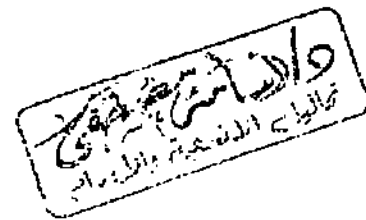
Microscopic Appearance:

Sections reveal a breast tissue infiltrated by moderately to poorly differentiated invasive ductal carcinoma composed of nests and cords of cells with ductal differentiation, surrounded by stromal desmoplastic reaction. Tumor cells are large, pleomorphic, with moderate to marked anaplasia and moderate mitotic activity. There is associated mild lymphocytic infiltrate. There is no associated significant intraductal component (DCIS).

Diagnosis:

LEFT BREAST MASS; TRU CUT BIOPSY:

- Invasive duct carcinoma, grade II-III.



Request No.	13004633
Request Date	13/05/2025
Verification Date	11/06/2025
Financial Category	اوراد دار السلام - نفقة التولية
Department	Out Patient

Registered by .marwa mohamed yousse

- ER: Positive (score 6/8).
- PR: Positive (score 6/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with small clusters
- Ki67: Positive in 50% of tumor cells

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within $\leq 10\%$ of the invasive tumor cells. IHC 3+ based on circumferential membrane staining that is complete, intense $> 10\%$ of tumor cells.

ISH groups:

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).

Verified by :

Dr. Elaa Mohamed

Printing Date : 18/06/2025 09:58:59

Page 2 of 3

Printed By : Mrs.Mayada Hegazy

Code: 22740
Sex: Female
التاريخ: 6- 5 - 2025م

اسم المريض: ولاء حامد علي ابراهيم
السن: 38 سنة
الطبيب المعالج: اد/ سوزان الحسين

Clinical Data: Left breast cancer.

Pathology report

Nature of the Specimen:

Paraffin block.

Gross Appearance:

Sections immunostained for ER, PR and Ki-67 using supersensitive monoclonal antibodies with Dako Envision (dextran substrate polymer) detection system. Prior microwave treatment for antigen retrieval and blocking of internal peroxidase was performed. DAB was utilized as a chromogen.

Microscopic Appearance:

ER: More than 80 % of tumor cells show moderate to strong nuclear staining (Allred scoring system: 8/8).

PR: More than 80 % of tumor cells show moderate to strong nuclear staining (Allred scoring system: 8/8).

Ki-67: 45 % of the tumor cells (at the hot spots) show strong nuclear staining.

Diagnosis:

ER, PR IMMUNOHISTOCHEMISTRY:

- **ER Status:** Positive (Allred scoring system: 8/8)
- **PR Status:** Positive (Allred scoring system: 8/8).

KI-67 IMMUNOHISTOCHEMISTRY:

- **Ki-67 labeling index:** 45 %.

د. أسامة مصطفى

Code: 22740
Sex: Female
التاريخ: 6- 5 - 2025م

اسم المريض: ولاء حامد علي ابراهيم
السن: 38 سنة
الطبيب المعالج: اد/ سوزان الحسين

Clinical Data: Left breast cancer.

Pathology report

Nature of the Specimen:

Paraffin block.

Gross Appearance:

Sections immunostained for c-erb-2 (HER2/neu) using supersensitive monoclonal antibodies with Dako Envision (dextran substrate polymer) detection system. Prior microwave treatment for antigen retrieval and blocking of internal peroxidase was performed. DAB was utilized as a chromogen.

Microscopic Appearance:

For the determination of HER2 protein overexpression, only the membrane staining intensity and pattern in the invasive part of tumor are evaluated. HER2 overexpression is interpreted as negative for both scores 0 (no staining is observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells) and 1+ (faint/barely perceptible incomplete membrane staining in $> 10\%$ of tumor cells). The test is interpreted as weakly positive (border line or equivocal) for score 2+ (circumferential membrane staining that is incomplete and/or weak/moderate within $> 10\%$ of the invasive tumor cells or complete and circumferential membrane staining that is intense and within $\leq 10\%$ of the invasive tumor cells). A strong positive result is interpreted when the score is 3+ (strong circumferential membrane staining in $> 10\%$ of invasive tumor cells).

Diagnosis:

IMMUNOSTAINING FOR C-ERB-2 (HER2/NEU)

- ***HER2/neu status: Strongly positive (score 3).***

[Handwritten signature]



جامعة المنزفة
مستشفى معهد الأورام
وحدة الطب النووي



Tc ⁹⁹m Bone scintigraphy

الاسم	ولاء حامد على ابراهيم	٢٩ سنة	
التاريخ	٢٠٢٥/٦/٢	النوع	F

TECHNIQUE:

Whole body anterior & posterior scans taken 3 hours after I.V injection. of (20) mci of the tracer Tc ⁹⁹m (MDP).

Finding:

- Increased radioactive tracer uptake over:
- Mid sternum .

OPENION.

- For clinical and radiological correlation .

Verified by:

Prof/Amira Ebrahim Aldesoky

Assistant Professor of clinical Oncology &
nuclear medicine

walaa hamed ali ibrahem DOB: 02-Jun-1986 ID: bone SEX: F
STUDY DATE: 02-Jun-2025

Patient Name: walaa hamed ali ibrahem
Study Name:

Patient ID: bone
Study Date: 02-Jun-2025

DOB: 02-Jun-1986

Wholebody 02-Jun-2025

Wholebody 02-Jun-2025



Anterior 634K Counts Duration:345sec



Posterior 662K Counts Duration:345sec



Anterior 634K Counts Duration:345sec



Posterior 662K Counts Duration:345sec



شعير الكوم - الرنين الغناطيسى المفلق : شعير الكوم - ش عبد الفتاح فرج متفرع من ش الأمين - برج الأنوار الحمدية
٠٤٨٢٣٣٣٣١٢ - ٠٤٨٢٣٣٣٣١٢ - ٠٤٨٢٣٣٣٣١٢ - ٠١٣٩٨٨-٥٢
الرنين المغناطيسى المفلق - PET- CT : شعير الكوم - ٢ ش الشهداء ميدان مجلس المدينة - ٠٤٨٢٣٣٣٧٤٩٤ - ٠١٠٠٤٢٧٧٤١٩
الأشعة المقطعية : شعير الكوم - ميدان شرف - ٨ ش الأمين - ٠٤٨٢٣٣٣١٩١ - ٠١٠٠٢١٨٥٥١
فرع مستشفى الجامعة برج حجازى شارع ياسين عبد الغفار بجوار مستشفى جامعة النوفية ٠٤٨ / ٢٢٢٥١١٤ - ٠١٠٣٢٤٧٤٧١٩

التاريخ : 2025/06/03

إسم المريض : ولاء حامد

C.T. EXAMINATION OF THE CHEST FOLLOWING INTRAVENOUS CONTRAST MEDIA ADMINISTRATION REVEALED:

- Left breast ill defined lesion measuring about 3.5x3.3 cm
- Normal lung parenchymal with no consolidation patches or ground glass opacities
- The trachea, main stem bronchi as well as their lobar and segmental divisions are normal.
- No evidence of mediastinal or hilar nodal enlargement.
- No pleural collections.
- Normal cardiac size and configuration.
- No detectable lesions in the chest wall.

C.T. EXAMINATION OF THE ABDOMEN AND PELVIS FOLLOWING INTRAVENOUS CONTRAST MEDIA ADMINISTRATION REVEALED:

- The liver is seen average in size and tissue density. It shows no focal or enhancing lesions.
- The intra and extra-hepatic biliary radicles are normal.
- The pancreas and spleen are normal.
- No renal masses could be noted
- Right and left supra renal glands are normal.
- No CT evidence of enlarged abdominal or pelvic lymph nodes.
- No ascites.
- Scanned pelvis showed no masses.
- Calcular gall bladder is noted

- Dr. Shrief Ramadan, MD
- Dr. AyaElmaghraby

Patient Name	: الأستاذ / ولاء حامد على ابراهيم	Request No.	: 13004633
Patient No.	: 28604151700541	Request Date	: 13/05/2025
Path No.	: SR-692/25	Verification Date	: 28/05/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Her2: Equivocal (score 2+) for SISH.
- Ki67: Positive in 50% of tumor cells.

• Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1 >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Rana Fathy

Dr. Heba Hasan

Verified by :

Dr.Toka Medhat

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Printing By : marwa mohamed youss



Patient Name :	الأستاذ / ولاء حامد على ابراهيم	Request No. :	13004633
Patient No. :	28604151700541	Request Date :	13/05/2025
Path No. :	SR-692/25	Verification Date :	11/06/2025
Sex :	Female	Financial Category :	اورام دار السلام - نفقة الدولة
Age :	39 Y	Department :	Out Patient
Referring Doc. :		IHC No. :	
		Registered by :	marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Left breast cancer.

Nature of Specimen: Referred block (Tru-cut biopsy).

Gross Examination:

- One referred block labelled as (22740) was received.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and

hematoxylin as counter stain. The sections treated against:

- ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9), E-cadherin, SISH.

Microscopic Examination:

* Histological type: Invasive duct carcinoma.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade III (3+3+2).

* DCIS: Not detected.

* Angioinvasion: Not detected.

* Perineural invasion: Not detected.

* IPT:

- E-cadherin: Positive in tumor cells.

- ER: Moderate nuclear reaction in 50% of tumor cells (score 6/8).

- PR: Strong nuclear reaction in 20% of tumor cells (score 6/8).

- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).

- SISH: Amplified with small clusters.

- Ki67: 50% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Referred block (Tru-cut biopsy):

- Invasive duct carcinoma, Grade III.

IPT:

Verified by :

Dr.Duaa Mohamed

Printing Date : 18/06/2025 09:58:59

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Printed By : Mrs.Mayada Hegazy



Patient Name : الأستاذ / ولاء حامد على ابراهيم	Request No. : 13004633
Patient No. : 28604151700541	Request Date : 13/05/2025
Path No. : SR-692/25	Verification Date : 28/05/2025
Sex : Female	Financial Category : أورام دار السلام - نفقة التولية
Age : 39 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Slice revision

Gross Examination:

- One paraffin block labeled by (22740) was received
- Sections prepared from paraffin block for immune-staining Using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
- * Ductal carcinoma in situ: No
- * Perineural invasion: No
- * Angioinvasion: No
- * IPT (Positive control showed appropriate reactivity):
- E-Cadherin: Positive.
- ER: Moderate nuclear reaction in 50% of tumor cells (score 6/8).
- PR: Strong nuclear reaction in 20% of tumor cells (score 6/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
- Ki67: 50% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade III.

IPT:

- ER: Positive (score 6/8).
- PR: Positive (score 6/8).

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Dr.Toka Medhat

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Page 1 of 2

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جمهورية مصر العربية
بطاقة تحقيق الشخصية

ولاء
حامد على إبراهيم طعيمة
شارع مسجد النور - زويز
مركز شبين الكوم - المتوقفة

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حاصلة على بكالوريوس علوم
شعبة كيمياء
أنثى
مستلمة
ضياء احمد رمضان احمد
البطاقة سارية حتى ٢٠٣١/١٠/١١

١٢

٢٠٢٤/١٠ ٢٨٦٠٤١٥١٧٠٠٥٤١

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